

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

November 17, 2025

MEMORANDUM FOR:

(b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE
Assistant Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

THROUGH:

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CDR (b)(6),(b)(7)(C),(b)(7)(F) RN
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FROM:

LCDR (b)(6),(b)(7)(C),(b)(7)(F) RN
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ICE Health Service Corps

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Investigator
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SUBJECT:

Mortality Review – Report of Findings
Chaofeng, GE, (b)(6),(b)(7)(C)

Executive Summary:

On August 5, 2025, the U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of alien Chaofeng GE, (b)(6),(b)(7)(C). ICE detained Mr. GE, a 32-year-old Chinese male, at Moshannon Valley Processing Center (MVPC), in Philipsburg, Pennsylvania (PA), from July 31 to August 5, 2025, the date of his death. An autopsy ruled Mr. GE's manner of death was suicide, and his cause of death was hanging.

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Prior to ICE custody, on November 22, 2023, the U.S. Border Patrol (USBP) arrested Mr. GE, near Tecate, California (CA), for entering the United States without admission or parole by an immigration officer. On November 25, 2023, USBP served Mr. GE a notice to appear, form I-862, charging inadmissibility pursuant to Section 212(a)(6)(A)(i) of the Immigration and Nationality Act, as an alien present in the United States without admission or parole. On the same date, USBP released Mr. GE on an Order of Recognizance, Form I-220A. On January 23, 2025, Lower Paxton Township, PA, police arrested Mr. GE on charges of conspiracy, theft by deception, criminal use of a communication facility, and unlawful use of a computer. ICE Enforcement and Removal Operations (ERO) Philadelphia lodged an Immigration Detainer Notice of Action, Form I-247A, with the Dauphin County Prison the next day. On July 31, 2025, a court, in Dauphin County, PA, convicted Mr. GE of accessing a device issued to another who did not authorize use, and sentenced him to six to 12 months incarceration, with credit for time served. On the same date, ERO Philadelphia arrested Mr. GE and detained him at MVPC.

On August 1, 2025, a licensed practical nurse (LPN) completed Mr. GE's intake screening using a Mandarin interpreter, documented he arrived from another facility without a transfer summary, his denial of past medical or mental health conditions, his denial of current or past thoughts of suicide or self-harm, and no current behavioral risks for suicide, and released him to general population. An advanced practice provider (APP) received and initialed Mr. GE's transfer summary, which contained documentation of a past suicide attempt, his diagnosis of unspecified affective disorder, and his use of an antidepressant.

On August 5, 2025, at 5:20 a.m., while conducting unit count, two detention officers (DOs) found Mr. GE, bound at the hands and feet, hanging in the handicap shower stall in C-Unit, pod 4. The DOs left the scene to inform the control officer, and the control officer called the medical emergency. Two medical staff members responded within four minutes, ordered Mr. GE's cut down from the shower stall, immediately initiated cardiopulmonary resuscitation (CPR), applied an automated external defibrillator (AED), and followed the prompts (no shock advised). At 5:31 a.m., custody staff called emergency medical services (EMS). At 5:44 a.m., EMS arrived and took over life saving measures; however, EMS staff were unable to resuscitate Mr. GE.

At 6:03 a.m. a Clearfield County, PA, coroner pronounced Mr. GE deceased.

Mortality Finding:

As a result of Mr. GE's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures or practices are warranted; and identify issues requiring further study.

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A mortality review committee (MRC) determined MVPC did not provide Mr. GE medical care within the safe limits of practice. Specifically, the MRC determined: 1) MVPC medical staff did not receive Mr. GE's transfer summary at intake or take appropriate action upon receipt of his transfer summary. 2) Upon discovering Mr. GE hanging in the shower stall, custody staff delayed life-saving measures. 3) Medical staff did not deliver effective CPR or demonstrate complete knowledge of CPR guidelines, and MVPC's policies and practices do not delineate staff responsibilities or require medical staff to respond with all emergency equipment. 4) Custody staff did not complete timely or thorough routine checks in Mr. GE's housing unit.

The MRC convened on October 24, 2025, and consisted of the following IHSC members: Dr. (b)(6),(b)(7)(C),(b)(7)(F) clinical director, physician reviewer; Lieutenant Commander (LCDR) (b)(6),(b)(7)(C),(b)(7)(F) nurse practitioner, APP reviewer; Captain (CAPT) (b)(6),(b)(7)(C),(b)(7)(F) eastern regional nurse manager, nurse reviewer; and LCDR (b)(6),(b)(7)(C),(b)(7)(F), psychologist, behavioral health clinical consultant, behavioral health provider reviewer.

The following summary of events was based on: 1) Mr. GE's MVPC medical records and EMS report; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. GE's MVPC detention file; 5) Mr. GE's autopsy report; 6) video footage from August 4 – 5, 2025; 7) an on-site review and staff interviews conducted by LCDR (b)(6),(b)(7)(C),(b)(7)(F), LCDR (b)(6),(b)(7)(C),(b)(7)(F), and LCDR (b)(6),(b)(7)(C),(b)(7)(F) at MVPC, on September 2 – 5, 2025; 8) applicable MVPC policies and procedures; and 9) ICE Detention Standards.

Summary of Events

On August 1, 2025, an LPN completed Mr. GE's intake assessment using a Mandarin language interpreter, documented he arrived from another facility without a transfer summary, his denial of any past medical or mental health conditions, no medication usage, his denial of current or past thoughts of suicide or self-harm, no current behavioral risks for suicide, and released him to general population.

- On the same date, an APP received, reviewed, and signed off on Mr. GE's transfer summary that included history of a suicide attempt and use of an antidepressant. The APP did not evaluate Mr. GE or refer him to behavioral health.

On August 5, 2025, at approximately 5:20 a.m., while conducting unit count, two MVPC DOs found Mr. GE, bound at the hands and feet, hanging in the handicap shower stall in C-Unit, Pod 4. The DOs left the scene to alert a DO in control, who immediately called a medical emergency via his radio, and additional MVPC staff responded.

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- At approximately 5:25 a.m., MVPC staff used a cut down tool to remove the bed sheet from around Mr. GE's neck, placed him on the floor, and began life saving measures, including CPR and applying an AED due to his lack of a pulse.
- At approximately 5:31 a.m., MVPC staff called 911 and requested EMS.
- At approximately 5:44 a.m., EMS staff arrived in C-Unit and took over life saving measures, including CPR.
- At approximately 6:03 a.m., EMS staff stopped life saving measures after the Clearfield County, PA, coroner pronounced Mr. GE deceased.

The final autopsy report revealed Mr. GE's cause of death was hanging. The coroner determined his manner of death was suicide.

LCDR (b)(6),(b)(7)(C) performed Mr. GE's psychological autopsy (procedure for analyzing the cause of death by reconstructing what the person thought, felt, and did before death, based on gathered information). Further specifics regarding the Behavioral Health Unit's findings and recommendations can be found within Mr. GE's psychological autopsy.

The MRC used the following accreditation standards for this review: ICE Performance-Based National Detention Standards (PBNDS), 2011 with 2016 revisions. The MRC identified the following program strengths, weaknesses, and recommendations during this review.

Strengths

The MRC identified the following strengths: 1) Medical staff responded to Mr. GE's medical emergency in under four minutes, initiated Mr. GE's cutdown, started CPR, applied the AED, prompted the 911 call, and guided other staff to administer chest compressions. 2) On August 5, 2025, an MVPC facility manager performed an after-action review and immediately implemented corrective actions to rectify findings from the internal investigation. MVPC's corrective actions included: implementing a new process to ensure medical staff receive and review transfer summaries at the time of intake; ensuring cutdown tools are readily accessible by placing them in the emergency bag and requiring custody supervisors to carry a cutdown tool; and changing the shower hooks to hooks with a three-pound load bearing limit. 3) MVPC has an in-house, board certified, clinical psychologist and a well-staffed behavioral health department.

Weaknesses

1. Inappropriate mental health care.

- On August 1, 2025, Mr. GE arrived to MVPC with a transfer summary; however, medical

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staff did not receive the transfer summary during his intake, and the intake nurse placed Mr. GE in general population after he denied a history of suicidality. Several hours after his intake, an APP received, reviewed, and signed off on Mr. GE's transfer summary that included history of a suicide attempt and use of an antidepressant, but the APP did not take appropriate action to assess or refer him for an assessment to determine the suitability of his housing classification.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.6, Significant Self-harm and Suicide Prevention and Intervention; section B, *Identification*.
- PBNDS 2011 with 2016 Revisions: Part 4.6, Significant Self-harm and Suicide Prevention and Intervention; section C, *Referral*.
- MVPC policy: 1-2, Medical and Mental Health Services, *Intake Screening Procedure*.

2. Delay of care.

- Custody staff who found Mr. GE did not immediately relieve the pressure off his neck and cut him down, activate an emergency response, or call emergency medical services.
- Custody staff did not have immediate access to a cut down tool, causing a delay in cutting Mr. GE down from the shower hooks.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section T, *Emergency Medical Services and First Aid*.
- PBNDS 2011 with 2016 Revisions: Part 4.6, Significant Self-harm and Suicide Prevention and Intervention; section H, *Intervention*.
- MVPC policy: 3, *General Post Orders*.
- MVPC policy: 10, *Suicide Recognition and Prevention*.
- MVPC policy: 1-3, Medical and Mental Health Services, *Emergency Services*.

3. Inappropriate emergency care.

- Medical staff keep emergency medications in a locked box, separate from other emergency equipment, and do not always take medications to the emergency response, risking a delay in emergency care.
- Custody staff delivered chest compressions at a rate exceeding the recommended rate, not allowing for effective chest recoil, rendering the chest compressions ineffective.
- Medical staff did not consistently deliver rescue breaths at the recommended rate and removed the facemask delivering oxygen between rescue breaths.

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- Medical staff lacked knowledge of basic life support compression and respiratory ratios and oxygen delivery during CPR, leading to ineffective CPR delivery.
- MVPC's policy does not clearly articulate health staff's responsibilities during an emergency response.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section T, *Emergency Medical Services and First Aid*.
- MVPC policy: 1-3, Medical and Mental Health Services, *Emergency Services*.

4. Staff training.

- Custody staff did not perform timely and thorough rounds in Mr. GE's housing area according to policy and did not check the shower area of the housing unit during each unannounced round.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.6, Significant Self-harm and Suicide Prevention and Intervention; section A, *Staff Training*.
- PBNDS 2011 with 2016 Revisions: Part 2.4, Facility Security and Control; section A, *Detainee Supervision and Monitoring*.
- PBNDS 2011 with 2016 Revisions: Part 2.9, Post Orders; section V, *Expected Practices*.
- MVPC policy: 3, *General Post Orders*.
- GEO policy: 11-12, *Facility Security and Control*.

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director (DAD) of Health Care Compliance (HCC).
- The IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, and MVPC's facility administrator and health authority, for review and any action(s) deemed appropriate.
- The respective IHSC HCC unit and ICE will ensure the corrective action plan, if applicable, is implemented and sustained.

End of report.